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Page: 2/2

From: Therapedia, LLC

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Revised 10/2021



17516 Matany Rd, #100

Justin, TX 76247

THERAPY ORDERS

Phone 940-654-4011

Fax 844-440-2104

Child's Name AIDAN LOEWEParent/Guardian Name JENNIFER STARNESDOB 12/01/2020Address 1637 18TH ST. NORTHLAKE, TX, 76226Physician Name DR. CARRIE JONESPhone: 214-325-8111Physician Phone 469-405-6638

Address _____

Precautions _____

☒ **Physical Therapy**☐ **Speech Therapy**☒ **Occupational Therapy**☐ Evaluate & Recommend☐ Evaluate & Recommend☐ Evaluate & Recommend☒ Evaluate & Treat☐ Evaluate & Treat☒ Evaluate & Treat

Diagnosis Code #

Diagnosis Code #

Diagnosis Code #

Q90.9
Down SyndromeQ90.9
Down Syndrome

Physician's Signature (required)

Carrie Jones, MD
812 US 377 N, Suite 100
Argyle, Texas 76226
(469) 405-6638

Date

4/9/2025**DIAGNOSIS CODES**

Listed below are some of the commonly used diagnoses for conditions that might be applicable for your patients with special needs. This list is **not all-inclusive**. Refer to the ICD 10 Manual for special codes, definitions, and exclusions that may apply to your patient.

PT Codes**ST Codes****OT Codes**

Abnormality of Gait, unspecified R26.9
Abnormality of Gait, other R26.89
Autism F84.0
Cerebral Palsy, unspecified G80.9
Congenital Hemiplegia G80.2
Quadriplegia G80.0
Lack of Coordination R27.8
Delayed Milestone in childhood R62.0
Diplegic CP, spastic G80.1
Down Syndrome Q90.9
Dystonia G24.8
Failure to Thrive R62.51
Spastic Hemiplegia unspcfd side G81.10
Generalized muscle weakness M62.81
Lack of Normal Physiologic Dev R62.50
Scoliosis M41.20
Torticollis M43.6
Spastic diplegic cerebral palsy G80.1
Unspcfd Symptoms involving:
-Nervous System R29.90
-Musculoskeletal System R29.91
Pain, unspecified G89
Contracture of muscle M62.4
Other muscle spasm M62.838
Extreme immaturity of newborn P07.1
Plagiocephaly Q67.3
Difficulty walking, not else classified R26.6
Abnormal Posture R29.3

Other Speech disturbance R47.89
Speech disturbance (unspcfd) R47.9
Childhood onset fluency disorder F80.81
stuttering
Dev delay speech/lang unspcfd F80.9
Expressive lang disorder F80.1
Mixed receptive-expressive lang F80.2
Other dev disorder of speech/lang F80.89
Phonological disorder F80.0
Social pragmatic comm disorder F80.82
Speech lang delay due to hearing loss F80.4
Cleft palate Q35
Cleft palate with lip Q37
Aphasia R47.01
Apraxia R48.2

Lack of coordination R27.8
Muscle Weakness M62.81
Visual disturbance H53.8
Convergence insufficiency H51.11
Sx of musculoskeletal system R29.91
Sx of nervous system R29.90
Disturbance of skin sensation R20.9
Congenital hypotonia P94.2
Sx/signs w/general sensations & perceptions R44.8
Lack of normal physiological dev in childhood (unspcfd) R62.50
Hyperesthesia R20.3
Sx/signs involving cognitive functions & awareness R41.89
Other

Feeding Codes

ARFID F50.82
Tongue tie Q38.1
Feeding Newborn P92
Infant feeding disorder F98.2
Pedi feeding disorder acute (< 3mths) R63.31
Pedi feeding disorder chronic (> 3mths) R63.32
Picky Eater R63.39
Feeding difficulties, unspcfd R63.30
Aspiration pharyngeal dysphagia R13.13
Dysphagia oral phase R13.11
Aspiration of fluid Y84.4

Physical Therapy Evaluation

Client Name: LOEWE, AIDAN**File #:** LOEAI0524**Document Date:** 7/1/2025**Date of Birth**

12/1/2020

Age

4 years, 7 months

Gender

Male

Physician

Dr. Carrie Jones, MD, #1386686459

Primary Insurance

American Therapy Administrators

Primary ID

868844-011-00002

HISTORY AND BACKGROUND

Diagnosis Codes

Code	Description
Q909	Down's syndrome, unspecified
M6281	Muscle weakness (generalized)

Date/Age of onset of current condition, concern or diagnosis:

Aidan is 4 y/o male presenting to the physical therapy re-evaluation with present concerns of muscle weakness secondary to Down's syndrome.

Parent's Concerns

Mom reports the following concerns and would like to continue services to address:

1. muscle weakness
2. independent stair climbing
3. decreased core strength

Introduction and Background

Aidan is a 4 year, 7 month old male with Trisomy 21 diagnosis. He lives at home with his mom, dad, and younger brother (2 years old). He has previously received OT, PT, and speech school based services, and is currently receiving speech and occupational therapy at Therapedia, LLC at this time. Per parental report, Aidan was rolling over at 3 months old, sitting independently at 11 months old, crawled at 14 months, and walked at 24 months. Mom is present during the subjective history portion of the re-evaluation and serves as a historian.

Past medical history and comorbidities:

Birth history:

- Born at 40 weeks gestation
- vaginal birth
- Time in the NICU: 6 days

Medical History:

- Down syndrome
- Tetralogy of Fallot surgery on 5/17/2021, no complications post-operatively per dad's report.

Allergies

no known allergies per parent report

Current Medications

prescribed medications

Medication Additional Information

Xyzal- allergies

Lansoprazole -acid reflux
Flonase-allergies

Safety Risks / Safety Concerns

supervision due to age

Primary Language

English

Other services involved in the care of this child

☒ Occupational Therapy ☒ Speech Therapy

Other Services Additional Information

Currently in occupational and speech services at Therapedia, LLC.

CURRENT FUNCTIONAL STATUS

Adaptive Equipment

orthotics

Equipment details

Bilateral SMOs.

Functional mobility

moves independently within the environment

Communication

☒ uses alternative communication devices /

Behavioral Observations

☒ object preoccupation / self stim behaviors ☒ structure and cueing required for participation in tasks

☒ decreased eye contact

☒ impulsive / lacks inhibition

☒ decreased safety awareness

☒ play skills appear to be below age level

☒ difficulty interacting with peers ☒ decreased attention to tasks

SENSORY SYSTEM

Sensory Systems Review

Description	Vision	Hearing	Tactile	Taste	Smell	Proprioception	Vestibular
Concerns (see below)						X	

Proprioception

Proprioception is the sense that provides us with information of the position of our body parts in space and in regards to other objects. It allows for perception of joint position and movement, muscle force and effort. Input through this system can be calming and may be used to regulate our state of being. This system also provides positional awareness needed in order to maintain balance. The following concerns regarding proprioceptive awareness were noted during this evaluation. Aidan demonstrates difficulty with controlling his body and often will speed up movement to complete the task.

☒ difficulty imitating motion or learning new movement patterns

☒ difficulty moving self in relation to other objects / motor planning concerns

☒ poor posture or slouching

Physical Therapy Evaluation

Client Name: LOEWE, AIDAN

Birthdate: 12/1/2020

[X] relies heavily on visual input during movement

[X] frequent falls or stumbling

INTEGUMENTARY

[X] No known concerns identified at this

CARDIOVASCULAR & PULMONARY SYSTEMS

[X] no known concerns identified at this time

MUSCULOSKELETAL SYSTEM

Strength

not specifically tested due to age and/or ability of client

Comments regarding strength

Strength was observed through functional observation by therapist. He continues to sit with a wide base of support and sacral sitting posture. Aidan demonstrates improved LE strength as he is now able to walk up the stairs with 1 hand on the railing with a step-to pattern. Aidan prefers to use the left leg leading when performing stairs due to the right leg being more weak. During descending stairs, Aidan demonstrates increased trunk rotation and requires 2 hand assistance indicating decreased lower extremity eccentric strength and control.

Range of Motion

see comments below for details

Comments regarding range of motion

Observed global hypermobility and flexibility during play. Aidan demonstrates mild improvement of neutral foot alignment after ~6 months of wearing his bilateral SMOs, however he continues to have increased pronation in standing.

[X] Posture

[X] Alignment or postural differences as detailed

Comments regarding posture

In sitting, Aidan continues to demonstrate a posterior pelvic tilt with a wide base of support. With cueing, Aidan is able to maintain ring sitting once positioned.

In standing, Aidan stands with a wide base of support with feet slightly wider than shoulders.

Pain

No complaints of pain as reported by mom.

NEUROMUSCULAR SYSTEM

Muscle Tone

Tone quality
decreased tone

Tone distribution

Movement impact

Muscle tone discussion

Coordination

coordination is moderately impacted

Coordination discussion

Aidan requires 1 hand assistance to maintain balance and lift foot to kick a ball. He is able to roll a ball in sitting, however does not catch the ball when thrown to him. He will catch a medium sized balloon 2/5 attempts. Aidan jumps with 2 footed takeoff and landing in free space with no UE support with full foot clearance for 3/5 attempts today. When throwing, Aidan throws with an extended arm laterally.

Balance

balance is moderately impacted

Balance discussion

Aidan stands with a wide base of support, however demonstrates improvement with maintaining arms at side during gait. Aidan is able to step onto and off of a four inch foam mat with no upper extremity support and maintain his balance. Aidan requires 1 hand assistance with lifting his foot for any single leg balance activities.

POSTURAL CONTROL AND MOVEMENT

Sitting

See posture.

Standing

See posture.

Gait/Ambulation

[X] wears or needs orthotics [X] ambulates on level surfaces

[X] ambulates on uneven surfaces [X] ambulates on stairs

[X] demonstrates atypical gait pattern

TESTS AND MEASURES

Peabody Developmental Motor Scales

The PDMS-2 is an individually administered test that assesses the motor functioning of children from birth to six years of age. The test is standardized and provides age specific normative data. The test is not predictive of future outcome. The areas evaluated include:

*Reflex--ability to automatically react to environmental events at an age appropriate level. Reflexes typically become integrated by the age of 12 months.

*Stationary--ability to sustain control of his/her body within its center of gravity.

*Locomotion--ability to move from one place to another (crawling, walking, running, hopping, jumping).

*Object manipulation--ability to manipulate balls (catching, throwing, kicking).

*Grasping--ability to use hands,

*Visual Motor Integration--ability to use visual perceptual skills to perform complex eye-hand coordination tasks.

PDMS-2 (gross motor)

Description	Raw Score	Standard Score	Percentile	Age Equivalent	Description
Reflexes					
Stationary	41	5	5th	33 months	Poor
Locomotion	95	3	1st	20 months	Very Poor
Object Manipulation	10	2	<1st	16 months	Very Poor
GROSS MOTOR QUOTIENT	sum: 10	quotient: 57			Very poor

PDMS-2 Discussion

Aidan's Peabody scores are limited by his inability to maintain his balance in single leg stance without assistance and maintaining tandem stance for ~3 seconds.

DAYC-2 (Developmental Assessment of Young Children: Second Edition)

Domain	Tested	Report Attached	ZScore	Descriptive Rating	Services Indicated
Cognitive	[X]	[X]	-3.33	Very Poor	Yes
Adaptive					
Communication					
Social					

Emotional					
Gross Motor	[X]	[X]	-3.13	Very Poor	Yes
Fine Motor					
Total Motor					

DAYC-2 Discussion

The DAYC-2 is a standardized developmental test used to assess functional skills in several domains including: cognitive, adaptive, communication, social emotional, gross motor, fine motor, and total motor.

Aidan is able to ascend and descend stairs, however he requires verbal cueing for maintaining a reciprocal pattern consistently. Descending stairs he continues to requires UE support.

CLINICAL ASSESSMENT

Strengths

- engages with toys
- independent ambulator
- supportive family
- active child

PT Areas of Concern

Impairments of Body Functions and Structures

- muscle weakness (core and lower extremities)
- postural control
- balance deficits
- coordination deficits
- standing alignment

Activity Limitations and Participation Restrictions

- developmental delays
- play skills and keeping up with his peers/younger brother

Clinical Presentation

The patient's clinical presentation is consistent with their diagnosis and good progress toward goals is anticipated.

Therapist's comments / Assessment

Aidan is a 4 year, 7 month old male presenting to the physical therapy re-evaluation for continued services to address concerns of core and leg strength, balance deficits, coordination deficits, and developmental delay. Aidan has met 2 out of 9 goals since the initial plan of care and has made good progress towards the remainder of goals which will be carried over into the next plan of care along with newly listed goals. Over this plan of care, Aidan has seen improvements in his ability to jump with 2 footed takeoff and landing and his midrange squatting form. Aidan continues to have difficulty with single leg balance activities and coordination deficits. These continued deficits greatly impact his functional mobility and gross motor play putting him at high risk for falls and injuries that could be sustained from falling. Skilled physical therapy interventions continue to be necessary at this time in order for him to achieve the listed functional goals. If physical therapy services are not rendered, it could result in further delays for functional mobility, play, and serious injuries sustained from falls.

PLAN OF CARE

Recommendations

Direct skilled physical therapy services are recommended based upon good potential for attainment of the listed goals.

<u>Treatment indicated</u>	<u>Frequency per Week</u>	<u>Duration</u>
Yes	1-2x/wk	12 months

Recommendations Discussion

Recommending 30-45 minute sessions over a period of 12 months.

Physical Therapy CPT Codes

[X] 97110 Ther Ex [X] 97112 Neuro Re-ed

[X] 97116 Gait Training

[X] 97140 Manual Therapy

[X] 97161, 97162, 97163 PT Eval

[X] 97164 PT Re-eval

[X] 97530 Ther Activities

[X] 97535 Self Care/Home Mgmt

[X] 97750 Physical Test and

[X] 97760 Orthotics management

[X] 97763 Orthotics subsequent

Goals

Description	Target %	Term	Goal Began	Met %	Comments	Today	Met	Met Date
Aidan will hold tall kneeling to play with bilateral upper extremities for 30 seconds to improve his core strength to participate in play with his brother.		Long Term	7/18/2024		GOAL CONTINUES	[X]		
Aidan will ambulate across an 8 foot balance beam without stepping off on 2/3 trials in order for him to safely ambulate between 2 cars parked closely together.		Long Term	7/18/2024		GOAL CONTINUES; Requires 2 HHA for maintaining tandem stance.	[X]		
Aidan will be able to kick a ball with both feet 3/5 attempts to be able to keep up with same age peers.		Long Term	7/18/2024		GOAL CONTINUES; Requires 1 hand assistance for balance when lifting foot to kick a ball.	[X]		
Aidan will utilize half kneeling with left and right lower extremities 2/3 attempts without the use of upper extremity support in order to improve in stability and safety when moving from the floor to a standing position.		Long Term	7/18/2024		GOAL CONTINUES;Half kneel to stand facilitation, prefers RLE >LLE.	[X]		
Aidan will be able to clear foot when jumping 3/5 attempts to improve coordination to be able to participate with peers at school.		Long Term	7/18/2024		GOAL MET. Jumps in free space 3/5 attempts with full foot clearance.	[X]	[X]	7/1/2025
Aidan will be able to stand his right leg for 5 seconds to improve balance to be able to play at daycare with his friends.		Short Term	7/18/2024		GOAL CONTINUES; Requires 1 HHA.	[X]		
Aidan will be able to stand his left leg for 5 seconds to improve balance to be able to play at daycare with his friends.		Short Term	7/18/2024		GOAL CONTINUES; Requires 1 HHA.	[X]		
Aidan will be able to stand on each leg for 10 seconds to improve balance deficits to keep up with same age peers.		Long Term	7/18/2024		GOAL DISCONTINUED; due to attention to the task.	[X]		
Aidan will demonstrate a reciprocal pattern during ascending and descending 4 stairs with 1 hand hold assistance to be able to navigate stairs at home safely.		Long Term	7/18/2024		GOAL CONTINUES; Ascends with 1 hand on railing and LLE leading. Descends with step-to pattern with 1 hand assistance from therapist.	[X]		
Aidan will perform 5 squats to improve lower body strength to improve transitions from floor to play.		Short Term	7/18/2024		GOAL MET.	[X]		7/1/2025

He will catch a medium size ball thrown from 5 feet away on 4/5 trials within 6 months in order to participate in ball play with peers.		Long Term	7/1/2025		NEW GOAL ADDED.	[X]		
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Provider Name: Olivia Kenyon, PT DPT

Olivia Kenyon, PT, DPT, NPI #: 1144069626

NPI #: 1144069626

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**Therapedia, LLC**

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PROGRESS NOTE

Client Name: **LOEWE, AIDAN**
File Number: **LOEAI0524**
Provider: **Olivia Kenyon, PT, DPT, NPI #: 1144069626**
Service Date: **8/12/2025** Visit #: **13**
Time In: **02:00 am** Time Out: **02:30 am**
Treatment Duration: **30 minutes**

Incident: PT

Gender: **Male** Referral Date: Physician: **Jones, Dr. Carrie**
Birth Date: **12/1/2020** Assessment Date: Phone: **(469) 405-6638**
Fax: **(940) 240-5517**

Diagnosis: **Q909 Down's syndrome, unspecified**
M6281 Muscle weakness (generalized)

Subjective:

Last Re-evaluation date: n/a
Annual evaluation date: 7/18/24
Prescription date: 7/1/24
Prescription renewal date: 7/1/25
Precautions/Considerations: supervision due to age

Mom waits in the lobby during the therapy session today. Mom reports noticing Aidan is keeping his arms at his side during walking more often.

Objective:

The patient engaged in therapeutic activities through functional tasks, play and games including:
-modified single leg balance when stomping on piggy bank coins; seeks out 1 HHA
-reciprocal stair navigation with minimal cueing for reciprocal pattern when ascending
-descending stairs with 1 hand on railing and RLE leading
-midrange squatting in free space with cueing for retrieving toys from knee height
-ring sitting on platform swing with 2 HHA and cueing for maintaining ring sitting

SEE GOALS FOR DETAILS

Assessment:

Therapeutic intervention is effective as demonstrated by progress toward goals (see goals for details). Aidan tolerated treatment well today. Aidan demonstrates improved core activation in sitting on the platform swing. He demonstrates improvement in arm positioning during gait and remains at side for ~60% of session. He continues to have difficulty with lateral weight shifts in standing to perform single leg stance with no UE support today. Aidan will continue to benefit from skilled PT services to improve the deficits listed above and to progress towards goals.

Plan:

Continue with current focus as per plan of care.
HEP: continued from the previous visit

Provider Name: Olivia Kenyon, PT DPT
Olivia Kenyon, PT, DPT, NPI #: 1144069626

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PROGRESS NOTE

Client Name: **LOEWE, AIDAN**
File Number: **LOEAI0524**
Provider: **Olivia Kenyon, PT, DPT, NPI #: 1144069626**
Service Date: **8/5/2025** Visit #: **12**
Time In: **02:00 am** Time Out: **02:30 am**
Treatment Duration: **30 minutes**

Incident: PT

Gender: **Male** Referral Date: Physician: **Jones, Dr. Carrie**
Birth Date: **12/1/2020** Assessment Date: Phone: **(469) 405-6638**
Fax: **(940) 240-5517**

Diagnosis: **Q909 Down's syndrome, unspecified**
M6281 Muscle weakness (generalized)

Subjective:

Last Re-evaluation date: n/a
Annual evaluation date: 7/18/24
Prescription date: 7/1/24
Prescription renewal date: 7/1/25
Precautions/Considerations: supervision due to age

Mom waits in the lobby during the therapy session today. Mom reports no new changes since the previous visit.

Objective:

The patient engaged in therapeutic activities through functional tasks, play and games including:
-modified single leg balance when stomping on piggy bank coins; seeks out 1 HHA
-reciprocal stair navigation with minimal cueing for reciprocal pattern when ascending
-descending stairs with 1 hand on railing and RLE leading
-lifting tunnel placing it upright for transverse abdominis activation
-catching a balloon with 2 hands from ~4 feet away
-sitting to tall kneeling in free space to reach overhead to place stuffed animals into bucket

SEE GOALS FOR DETAILS

Assessment:

Therapeutic intervention is effective as demonstrated by progress toward goals (see goals for details). Aidan tolerated treatment well today. Aidan demonstrates improvement in reciprocal stair navigation when ascending stairs with 1 HHA and minimal cueing. Performed sitting to tall kneeling in free space to improve glutes and core co-contraction to improve postural stability. Aidan will continue to benefit from skilled PT services to improve the deficits listed above and to progress towards goals.

Plan:

Continue with current focus as per plan of care.
HEP: continued from the previous visit

Provider Name: Olivia Kenyon, PT DPT
Olivia Kenyon, PT, DPT, NPI #: 1144069626

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PROGRESS NOTE

Client Name: **LOEWE, AIDAN**
File Number: **LOEAI0524**
Provider: **Olivia Kenyon, PT, DPT, NPI #: 1144069626**
Service Date: **7/31/2025** Visit #: **11**
Time In: **01:50 am** Time Out: **02:15 am**
Treatment Duration: **25 minutes**

Incident: PT

Gender: **Male** Referral Date: Physician: **Jones, Dr. Carrie**
Birth Date: **12/1/2020** Assessment Date: Phone: **(469) 405-6638**
Fax: **(940) 240-5517**

Diagnosis: **Q909 Down's syndrome, unspecified**
M6281 Muscle weakness (generalized)

Subjective:

Last Re-evaluation date: n/a
Annual evaluation date: 7/18/24
Prescription date: 7/1/24
Prescription renewal date: 7/1/25
Precautions/Considerations: supervision due to age

Mom waits in the lobby during the therapy session today. Mom reports no new changes since the previous visit.

Objective:

The patient engaged in therapeutic activities through functional tasks, play and games including:
-modified single leg balance when stomping on piggy bank coins; seeks out 1 HHA
-reciprocal stair navigation with minimal cueing for reciprocal pattern when ascending
-descending stairs with 1 hand on railing and RLE leading
-lifting tunnel placing it upright
-pushing tunnel against therapist resistance
-supine to sit transitions over tunnel with cueing

SEE GOALS FOR DETAILS

Assessment:

Therapeutic intervention is effective as demonstrated by progress toward goals (see goals for details). Aidan tolerated treatment well today. Aidan demonstrates improvement in reciprocal stair navigation when ascending stairs with 1 HHA and minimal cueing. When descending stairs, Aidan will laterally turn his body and lead with RLE. Performed stomping on the piggy to improve single leg balance. Aidan will continue to benefit from skilled PT services to improve the deficits listed above and to progress towards goals.

Plan:

Continue with current focus as per plan of care.
HEP: continued from the previous visit

Provider Name: Olivia Kenyon, PT DPT
Olivia Kenyon, PT, DPT, NPI #: 1144069626

NPI #: 1144069626
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PROGRESS NOTE

Client Name: **LOEWE, AIDAN**
File Number: **LOEAI0524**
Provider: **Olivia Kenyon, PT, DPT, NPI #: 1144069626**
Service Date: **7/10/2025** Visit #: **10**
Time In: **02:15 am** Time Out: **02:45 am**
Treatment Duration: **30 minutes**

Incident: PT

Gender: Male	Referral Date:	Physician: Jones, Dr. Carrie
Birth Date: 12/1/2020	Assessment Date:	Phone: (469) 405-6638
		Fax: (940) 240-5517

Diagnosis: **Q909 Down's syndrome, unspecified**
M6281 Muscle weakness (generalized)

Subjective:

Last Re-evaluation date: n/a
Annual evaluation date: 7/18/24
Prescription date: 7/1/24
Prescription renewal date: 7/1/25
Precautions/Considerations: supervision due to age

Mom waits in the lobby during the therapy session today. Mom reports no new changes since the previous visit.

Objective:

The patient engaged in therapeutic activities through functional tasks, play and games including:
-modified single leg balance when stomping on piggy bank coins; seeks out 1 HHA
-reciprocal stair navigation with minimal cueing for reciprocal pattern when ascending
-descending stairs with 1 hand on railing and RLE leading
-lifting weighted mat from the floor and tossing it over a line when ring sitting

SEE GOALS FOR DETAILS

Assessment:

Therapeutic intervention is effective as demonstrated by progress toward goals (see goals for details). Aidan tolerated treatment well today. Aidan demonstrates improvement in reciprocal stair navigation when ascending stairs with 1 HHA and minimal cueing. When descending stairs, Aidan will laterally turn his body and lead with RLE. He requires 1 HHA when performing single leg balance activities. Aidan will continue to benefit from skilled PT services to improve the deficits listed above and to progress towards goals.

Plan:

Continue with current focus as per plan of care.
HEP: continued from the previous visit

Provider Name: Olivia Kenyon, PT DPT
Olivia Kenyon, PT, DPT, NPI #: 1144069626

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PROGRESS NOTE

Client Name: **LOEWE, AIDAN**
File Number: **LOEAI0524**
Provider: **Olivia Kenyon, PT, DPT, NPI #: 1144069626**
Service Date: **7/1/2025** Visit #: **9**
Time In: **02:00 am** Time Out: **02:30 am**
Treatment Duration: **30 minutes**

Incident: PT

Gender: Male	Referral Date:	Physician: Jones, Dr. Carrie
Birth Date: 12/1/2020	Assessment Date:	Phone: (469) 405-6638
		Fax: (940) 240-5517

Diagnosis: Q909	Down's syndrome, unspecified
M6281	Muscle weakness (generalized)

Subjective:

See Re-eval.

Objective:

See Re-eval.

Assessment:

See Re-eval.

Plan:

See Re-eval.

Provider Name: Olivia Kenyon, PT DPT
Olivia Kenyon, PT, DPT, NPI #: 1144069626

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